



SurgiScan

DOCUMENT INTELLIGENCE FOR HEALTHCARE.

Per-doctor SaaS · Monthly page credits · POPIA-compliant · Built in South Africa

Prepared by: Luzuko Ndzuta

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Two pains every SA private GP has in common

We've sat in the rooms. The details vary; the pains don't. Whether the practice runs on paper, on an EHR, or on a fully wired-up workflow — every single GP we meet has the same two unsolved problems.

01 · Paper rooms

Every GP we meet has a back room of legacy files — whether they're paper-based, partially digital, or fully on an EHR. That archive is dead weight until it becomes structured data.

02 · No clinical analytics

Not one practice we've spoken to has a working view of their chronic disease cohorts, claims aging, or patient deterioration risk. The data exists; nobody's looking at it.

Three kinds of doctor — one universal pair of pains

What changes between practices is whether they have an EHR, and whether their workflow actually hangs together end-to-end. That tells us which Practice Platform tier (if any) they need. The Intelligence Layer they all need.

Type	Has EHR?	End-to-end workflow?	Legacy archive?	What we sell them
A	No	No	Yes	Practice Platform + Digitisation + Analytics
B	Yes	No	Yes	Practice Platform (upgrade) + Digitisation + Analytics
C	Yes	Yes	Yes	Digitisation + Analytics (no platform change)

How SurgiScan is sold

Two product lines. Buy one or both. The Intelligence Layer works on top of the Practice Platform — or alongside whatever EHR you already run.

PRACTICE PLATFORM	INTELLIGENCE LAYER
Buy this if you need an EHR or your workflow is broken.	Buy this if you have paper to digitise or want clinical analytics — even if your EHR isn't ours.
Two tiers: Essential — go digital end-to-end (no EHR today). Professional — consolidate your tool stack, add AI Scribe + telehealth.	Three modules, à la carte: Digitisation — monthly page credits. Advanced Analytics — the view of your practice you don't have. Clinical AI (Beta) — invite-only design partners.

The integration story (for doctors with their own EHR)

If you already run an EHR — TrakCare, GoodX, Healthbridge, or anything else — the Intelligence Layer plugs in two ways, your choice:

- API integration — real-time sync where your EHR exposes one. Patient records flow both directions.
- CSV / FHIR batch — scheduled exports from your EHR, ingested into our analytics layer. The end result is the same: structured data, ready to be searched, charted and reported on.

Either way, your existing EHR stays the system of record. SurgiScan is a layer that adds capability, not a replacement that takes work away from a tool you already paid for.

PRODUCT LINE 01

Practice Platform

An EHR + workflow system, built for South African private practice.

For doctors who don't have an EHR, or who have one but spend their evenings stitching together billing, video, dictation, and queue tools that don't talk to each other.

■ Essential — go digital end-to-end

For: GPs who have nothing digital today (Type A). Move the entire core workflow — registration, encounter, prescription, invoice — onto one platform from day one.

- Full Patient EHR — registry, vitals (auto-BMI), allergies, AI-coded ICD-10 diagnoses, NAPPI medications.
- Reception check-in and patient registry.
- Prescription writing with allergy interaction checks.
- Billing and invoicing with PayFast online payments.
- Audit log and role-based access (POPIA-aligned).

Not included: AI Scribe, telehealth, advanced analytics dashboards.

■ Professional — consolidate your stack

For: GPs who have an EHR but don't have a workflow (Type B). Replace the patchwork of tools with one system.

- Everything in Essential, plus:
- AI Scribe — drafts SOAP notes from your voice during the consult; you review and sign in 30 seconds.
- Telehealth — video calls, patient chat, e-prescriptions, sick notes, referral letters.
- Reception, queue and vitals stations (vitals auto-populate the encounter).
- Workflow dashboards — patient flow, no-show rates, doctor productivity.

Note: Advanced clinical analytics (chronic disease cohorts, claims aging, semantic search) and Clinical AI are sold separately as part of the Intelligence Layer — see next section.

PRODUCT LINE 02**Intelligence Layer**

Three modules. Sold à la carte. Works on SurgiScan or your existing EHR.

Every GP we meet — paper-based, EHR-based, or fully wired — has a back room of legacy files and no view of their clinical data. The Intelligence Layer solves both, regardless of which Practice Platform (if any) you run.

MODULE 01**Digitisation**

What it does: turns the room full of paper into structured patient records. LandingAI extraction reads any scanned medical document; a human-in-the-loop queue validates before anything hits the live system.

Output goes wherever you want it: into the SurgiScan EHR, exported to your existing EHR via FHIR / CSV / JSON, or both.

- LandingAI document extraction engine — handwritten + typed.
- Human-in-the-loop validation queue — yours or ours.
- Auto-population — allergies, diagnoses, vitals, medications routed to their correct tables.
- FHIR / CSV / JSON export to whatever EHR you already run.
- POPIA-compliant data residency throughout.

Pricing model: monthly page credits, not project fees

A typical practice has 20,000+ files at ~10 pages each — a R1M project bill nobody will sign. So we don't charge that way. You pay a monthly subscription for a fixed page allowance; we work through your archive at that pace. It takes years to finish — and that's the point. Predictable cost for you, predictable revenue for us, no project sticker shock.

Founder-plan precedent: R3,500 / month for 1,500 pages = **R2.33** per page (vs R5 / page rack rate).

MODULE 02

Advanced Clinical Analytics

What it does: the view of your practice you've never had. Chronic disease cohorts, claims aging, no-show patterns, doctor productivity, drug spend — all the questions you've meant to ask, answered.

Pulls from SurgiScan if that's your EHR; ingests from your existing EHR via API or scheduled CSV / FHIR if it isn't.

- **Chronic disease cohorts** — diabetes, hypertension, asthma — sliced by control status, age, geography.
- **Claims aging + no-show analysis** — where revenue is leaking, which medical aids pay slowest, which patients miss most.
- **Doctor productivity** — consults per session, time per encounter, billing variance, in group practices.
- **Unified semantic search** — natural-language queries across every record in your archive.
- Custom dashboards + scheduled email reports.

Designed to plug onto a digitised archive — Module 01 is the data pipeline, Module 02 is what you do with it.

MODULE 03 · BETA · INVITE-ONLY

Clinical AI

What it is: a Beta partnership programme, not yet a commercial product. We work with a small number of design-partner practices to develop and validate clinical decision support — calibrated diagnostics, safety monitoring, risk scoring, imaging AI.

Available only to practices on the SurgiScan Practice Platform (or whose data has been loaded into the SurgiScan analytics layer via Module 01). The data model has to be ours for the AI to work safely.

- **Diagnostic Coordinator** — ranked differential diagnoses with calibrated confidence.
- **Safety Monitor** — silently watches for MI, PE, sepsis, stroke; alerts before the patient leaves the room.
- **Risk scoring + survival curves** — 1, 5 and 10-year projections for chronic patients.
- **Imaging AI** — chest X-ray flagging with bounding boxes.
- **Predictive population health** — patients likely to deteriorate in the next 90 days.
- **Cryptographically-signed audit trail** — regulator-grade evidence of every AI inference.

Honest framing: this is clinical decision support, not autonomous prescribing. Doctor verifies every recommendation. No SAHPRA approval as a medical device. Beta scope is finite — high-prevalence SA primary care conditions first. We open commercial access once we have calibration data and case studies; for now, talk to us about being a design partner.

At a glance

Mix and match. Most practices buy one Practice Platform tier (or none, if they have an EHR) plus one or more Intelligence Layer modules.

Capability	Essential	Professional	Digitise	Analytics
Practice Platform				
Patient EHR + AI ICD-10 + NAPPI	✓	✓	—	—
Reception + Billing + PayFast	✓	✓	—	—
Allergy alerts in prescriptions	✓	✓	—	—
Audit log + role-based access	✓	✓	—	—
AI Scribe (notes from voice)	—	✓	—	—
Telehealth + e-prescriptions	—	✓	—	—
Reception / queue / vitals stations	—	✓	—	—
Intelligence Layer (works with SurgiScan or your existing EHR)				
Document digitisation engine	—	—	✓	—
Monthly page credits (subscription)	—	—	✓	—
Human-in-the-loop validation queue	—	—	✓	—
Export to JSON / CSV / FHIR	—	—	✓	✓
Chronic disease + cohort analytics	—	—	—	✓
Claims aging + no-show + productivity	—	—	—	✓
Unified semantic search across archive	—	—	—	✓
Ingest from your existing EHR (API/ CSV)	—	—	✓	✓

Clinical AI not shown — it's invite-only Beta and not commercially available yet. Talk to us if you want to be a design partner.

Why South African GPs choose SurgiScan

1. **Built here.** POPIA, HPCSA prescription requirements, NAPPI codes, ICD-10 SA edition (41,008 codes loaded), PayFast — all native to the platform, not bolted on.
2. **Digitisation as a subscription, not a project.** Most vendors quote a six-figure project fee for an archive nobody will sign. We charge a monthly page allowance instead — payable, predictable, and the basis of a multi-year relationship.
3. **Works with your existing EHR.** The Intelligence Layer ingests from any EHR via API or CSV / FHIR. You don't have to rip out a system that works.
4. **AI that explains itself.** Every recommendation comes with reasoning the doctor can override and audit the regulator can read.
5. **Multi-tenant by design.** Each practice's data is isolated; what happens at one practice never crosses to another.

What's not on this brochure

- **Final pricing.** We finalise per-module and per-tier pricing with each practice based on doctor count, page volume, and EHR integration scope. Contact us for a quote.
- **Other verticals.** Progno Labs also serves Mining, Logistics, Legal and Finance with the same underlying engine. Healthcare is our primary focus.
- **Roadmap items.** Medical-aid claims auto-submission (Discovery, BHF, COIDA), multi-site practice management, native integrations with major SA EHRs — all on the Phase 2 roadmap, not promised here.

Talk to us

Web	medicdata.co.za
Email	luzuko@medicdata.co.za
Phone	+27 (0) 76 969 5462
Demo	book at medicdata.co.za/demo